

Observation Behavioral Health Level of Care

ORG: B-906-OBS (BHG)

MCG Health
Behavioral Health
Care
28th Edition

This MCG guideline has not yet been updated to reflect new guidance and requirements found in the 2024 Medicare Advantage and Part D Final Rule (CMS-4201-F). For Medicare Advantage beneficiaries, before referring to MCG's guidelines users should first follow all guidance related to this topic located in CMS National Coverage Determinations (NCDs), Local Coverage Determinations (LCDs), and other statutes and regulations. Content used in MCG guidelines may be used to supplement but does not replace, modify, or supersede existing Medicare regulations or applicable NCDs or LCDs.

- Admission Guidelines
- Recovery Course
- Discharge Guidelines
- References
- Footnotes
- Definitions

Admission Guidelines

- Admission to **Observation Level of Care** is indicated due to **ALL** of the following^[A](4)(10)(11)(12)(13):
 - Care needed requires further evaluation, treatment, or disposition arrangement for behavioral health issues, as indicated by **1 or more** of the following(14)(15)(16)(17):
 - Imminent danger to self for adult or Imminent danger to self for child or adolescent
 - Imminent danger to others for adult or Imminent danger to others for child or adolescent
 - **Severe** dysfunction in daily living for adult or **Severe** dysfunction in daily living for child or adolescent
 - Patient is experiencing withdrawal or other drug-induced disorder.^[B]^[C](3)(23)(24)(25)(26)(27)  CIWA-Ar Calculator  COWS Calculator
- ☐ Observation care is appropriate for anticipated scope and care needed for patient, as indicated by **ALL** of the following:
 - Care is beyond scope of usual outpatient care episode or symptoms are not adequately controlled despite emergency department care.^[D]
 - Care needed is appropriate for observation care, as indicated by **1 or more** of the following(10)(14)(28):
 - Diagnostic evaluation is needed and is not feasible in outpatient setting (eg, awaiting psychiatric evaluation which is not available in ambulatory care, or outpatient evaluation is inappropriate to patient condition).
 - Acute treatment and response evaluation is needed (eg, acute medication and observation for exacerbation of chronic psychosis).
 - Monitoring is needed while awaiting suitability for evaluation (eg, patient cannot be adequately assessed because of acute intoxication or substance-induced effect) or psychosocial management and arrangement for follow-up care.
 - Biopsychosocial stressors^[E] potentially contributing to clinical presentation (eg, comorbidities,^[F]^[G] illness history, environment,^[H] social network, ability to cope, and level of engagement^[I]) have been assessed and are absent or manageable at proposed level of care.(1)(2)(3)(4)(5)(6)(7)(9)(35)(36)

Recovery Course

Stage	Clinical Status	Interventions	Evaluation
-------	-----------------	---------------	------------

1	• Clinical Indications met[J] • Social Determinants of Health Assessment • Begin Discharge planning	• Appropriate crisis management instituted • Begin Care coordination • Regular safety checks done at appropriate frequency	• Symptom and functioning assessment at appropriate frequency documented • Physician evaluation of patient and progress performed • Review of need for medication or adjustment
2	• Social Determinants of Health Assessment • Further care using crisis intervention, treatment at another level of care, or discharge to outpatient follow-up identified as appropriate		

(4)(29)(36)

Recovery Milestones are indicated in **bold**.

Discharge Guidelines

- Discharge from observation level of care is appropriate for[K][L] **1 or more** of the following:
 - Continued intervention with goal of short-term stabilization is appropriate; see Crisis Intervention Behavioral Health Level of Care [BHG](#).
 - Evaluation indicates need for admission to specific behavioral health care setting; see appropriate Behavioral Health Care guideline.
 - Maintenance care or as needed follow-up is indicated by **ALL** of the following:
 - Sufficient evaluation completed
 - Presenting signs, symptoms, and other findings resolved or sufficiently improved
 - Care requirements appropriate for outpatient follow-up

References

1. American Association of Community Psychiatrists. Level of Care Utilization System for Psychiatric and Addiction Services (LOCUS). Adult version 20 [Internet] American Association of Community Psychiatrists. 2016 Dec Accessed at: <https://www.communitypsychiatry.org/keystone-programs/locus>. [accessed 2023 Sep 11] [Context Link 1, 2, 3, 4, 5, 6, 7, 8, 9, 10]
2. American Association of Community Psychiatrists. CALOCUS/CASII: Child and Adolescent Level of Care Utilization System. Child and Adolescent Version 20 [Internet] American Association of Community Psychiatrists. 2019 Jul Accessed at: <https://www.communitypsychiatry.org/keystone-programs/locus>. [accessed 2023 Sep 08] [Context Link 1, 2, 3, 4, 5, 6, 7, 8, 9, 10]
3. Level of care placement. In: Mee-Lee D, Shulman GD, Fishman MJ, Gastfriend DR, Miller MM, Provence SM, editors. ASAM Criteria Treatment Criteria for Addictive, Substance-Related, and Co-Occurring Conditions. 3rd ed. Carson City, NV: The Change Companies; 2013:174-306. [Context Link 1, 2, 3, 4]
4. APA Work Group on Psychiatric Evaluation. The American Psychiatric Association practice Guidelines for the Psychiatric Evaluation of Adults. 3rd edition [Internet] American Psychiatric Association. 2016 Accessed at: <https://psychiatryonline.org/>. [accessed 2023 Sep 21] DOI: 10.1176/appi.books.9780890426760. [Context Link 1, 2, 3, 4, 5, 6, 7]
5. CASII. Child and Adolescent Service Intensity Instrument [Internet] American Academy of Child and Adolescent Psychiatry. Accessed at: https://www.aacap.org/aacap/Member_Resources/Practice_Information/CASII.aspx. [accessed 2023 Sep 25] [Context Link 1, 2, 3, 4, 5, 6, 7, 8, 9, 10]
6. Child & Adolescent Service Intensity Instrument (CASII) User's Manual version 4.0. American Academy of Child and Adolescent Psychiatry 2014 Oct. [Context Link 1, 2, 3, 4, 5, 6, 7, 8, 9, 10]
7. Early Childhood Service Intensity Instrument (ECSII) for Infants, Toddlers, and Preschool-aged Children Ages 0-5 version 1. American Academy of Child and Adolescent Psychiatry 2009 Sep. [Context Link 1, 2, 3, 4]
8. Assessment measures. In: American Psychiatric Association, editor. Diagnostic and Statistical Manual of Mental Disorders. DSM-5-TR ed. American Psychiatric Association; 2022:841-857. [Context Link 1]
9. Rosser J, Michael S. Partial Hospitalization Programs and Intensive Outpatient Programs. 2021 AABH Standards and Guidelines [Internet] Association for Ambulatory Behavioral Healthcare. 2021 Accessed at: <https://aabh.org/>. [accessed 2023 Sep 12] [Context Link 1, 2]
10. Kelly MP, Shapshak D. Thought disorders. In: Walls RM, editor. Rosen's Emergency Medicine: Concepts and Clinical Practice. 10th ed. Philadelphia, PA 19103-2899: Elsevier; 2023:1333-1337.e2. [Context Link 1, 2]
11. Iannone P, Lenzi T. Effectiveness of a multipurpose observation unit: before and after study. Emergency Medicine Journal 2009;26(6):407-14. DOI: 10.1136/emj.2007.057539. [Context Link 1] View abstract...
12. Alpern ER, Calello DP, Windreich R, Osterhoudt K, Shaw KN. Utilization and unexpected hospitalization rates of a pediatric emergency department 23-hour observation unit. Pediatric Emergency Care 2008;24(9):589-94. [Context Link 1] View abstract...

13. Soyka M, et al. The World Federation of Societies of Biological Psychiatry (WFSBP) guidelines for the biological treatment of substance use and related disorders. Part 2: Opioid dependence. *World Journal of Biological Psychiatry* 2011;12(3):160-187. DOI: 10.3109/15622975.2011.561872. (Reaffirmed 2022 May) [Context Link 1] View abstract...
14. Leykum LK, Huerta V, Mortensen E. Implementation of a hospitalist-run observation unit and impact on length of stay (LOS): a brief report. *Journal of Hospital Medicine* 2010 Nov-Dec;5(9):E2-5. DOI: 10.1002/jhm.642. [Context Link 1, 2] View abstract...
15. Young HW, Shapiro MA. Suicidal Behavior. In: Walls RM, editor. *Rosen's Emergency Medicine: Concepts and Clinical Practice*. 10th ed. Philadelphia, PA 19103-2899: Elsevier; 2023:1358-1365.e4. [Context Link 1]
16. Abraham G, Maher PJ. Delirium and dementia. In: Walls RM, editor. *Rosen's Emergency Medicine: Concepts and Clinical Practice*. 10th ed. Philadelphia, PA 19103-2899: Elsevier; 2023:1269-1280.e2. [Context Link 1]
17. Service planning and placement. In: Mee-Lee D, Shulman GD, Fishman MJ, Gastfriend DR, Miller MM, Provence SM, editors. *ASAM Criteria Treatment Criteria for Addictive, Substance-Related, and Co-Occurring Conditions*. 3rd ed. Carson City, NV: The Change Companies; 2013:105-126. [Context Link 1, 2]
18. Nelson LS, et al. The ASAM Clinical Practice Guideline on Alcohol Withdrawal Management. [Internet] American Society of Addiction Medicine. 2020 Jan Accessed at: <https://www.asam.org/>. [accessed 2023 Oct 09] [Context Link 1]
19. Sullivan JT, Sykora K, Schneiderman J, Naranjo CA, Sellers EM. Assessment of alcohol withdrawal: the revised clinical institute withdrawal assessment for alcohol scale (CIWA-Ar). *British Journal of Addiction* 1989;84(11):1353-1357. [Context Link 1] View abstract...
20. Clinical Institute Withdrawal Assessment for Alcohol (CIWA-Ar). [Internet] Merck Manual. Accessed at: <https://www.merckmanuals.com/professional/multimedia/clinical-calculator/ciwa-ar-clinical-institute-withdrawal-assessment-for-alcohol-scale>. [accessed 2023 Oct 20] [Context Link 1]
21. Wesson DR, Ling W. The Clinical Opiate Withdrawal Scale (COWS). *Journal of Psychoactive Drugs* 2003 Apr-Jun;35(2):253-259. DOI: 10.1080/02791072.2003.10400007. [Context Link 1] View abstract...
22. Clinical Opiate Withdrawal Scale. [Internet] National Institutes of Health National Institute on Drug Abuse. 2003 Accessed at: <https://www.drugabuse.gov/nidamed-medical-health-professionals>. [created 1999; accessed 2023 Oct 17] [Context Link 1]
23. Finnell JT. Alcohol-related disease. In: Walls RM, editor. *Rosen's Emergency Medicine: Concepts and Clinical Practice*. 10th ed. Philadelphia, PA 19103-2899: Elsevier; 2023:1846-1860.e2. [Context Link 1]
24. Addressing withdrawal management and intoxication management. In: Mee-Lee D, Shulman GD, Fishman MJ, Gastfriend DR, Miller MM, Provence SM, editors. *ASAM Criteria Treatment Criteria for Addictive, Substance-Related, and Co-Occurring Conditions*. 3rd ed. Carson City, NV: The Change Companies; 2013:127-173. [Context Link 1, 2]
25. Kleber HD, et al. Practice Guideline for the Treatment of Patients with Substance Use Disorders. 2nd ed. [Internet] American Psychiatric Association. 2007 Apr Accessed at: <https://psychiatryonline.org/>. [created 2006; accessed 2023 Sep 21] DOI: 10.1176/appi.books.9780890423363.141077. [Context Link 1]
26. Strain EC. Substance use disorders: introduction. In: Sadock BJ, Sadock VA, Ruiz P, editors. *Kaplan and Sadock's Comprehensive Textbook of Psychiatry*. 10th ed. Philadelphia, PA: Lippincott Williams & Wilkins; 2017:1262-1264. [Context Link 1]
27. Drug Misuse Opioid Detoxification. NICE Clinical Guidance CG52 [Internet] National Institute for Health and Care Excellence. 2007 Jul (NICE reviewed 2019) Accessed at: <https://www.nice.org.uk/guidance>. [accessed 2023 Aug 08] [Context Link 1] View abstract...
28. Ross MA, Graff LGt. Principles of observation medicine. *Emergency Medicine Clinics of North America* 2001;19(1):1-17. [Context Link 1, 2] View abstract...
29. Venkatesh AK, Geisler BP, Gibson Chambers JJ, Baugh CW, Bohan JS, Schuur JD. Use of observation care in US emergency departments, 2001 to 2008. *PLoS ONE* 2011;6(9):e24326. DOI: 10.1371/journal.pone.0024326. [Context Link 1, 2] View abstract...
30. Abadi M, et al. Achieving whole health: a preliminary study of TCMLH, a group-based program promoting self-care and empowerment among Veterans. *Health Education & Behavior* 2022;49(2):347-357. DOI: 10.1177/10901981211011043. [Context Link 1, 2, 3] View abstract...
31. Joshi H, Corcoran-Lozano P. The behavioral health consultant: roles and responsibilities. *Pediatric Clinics of North America* 2021;68(3):563-571. DOI: 10.1016/j.pcl.2021.02.002. [Context Link 1, 2, 3] View abstract...
32. Rosenbaum E, Gordon AE, Cresta J, Shaughnessy AF, Jonas WB. Implementing whole person primary care. *Annals of Family Medicine* 2023;21(2):Online. DOI: 10.1370/afm.2952. [Context Link 1, 2, 3] View abstract...
33. Pourat N, et al. Final Evaluation of California's Whole Person Care (WPC) Program. [Internet] UCLA Center for Health Policy Research. 2022 Dec Accessed at: <https://healthpolicy.ucla.edu/publications/Pages/default.aspx>. [accessed 2023 Sep 08] [Context Link 1, 2, 3]
34. Matching multidimensional severity and level of function with type and intensity of service. In: Mee-Lee D, Shulman GD, Fishman MJ, Gastfriend DR, Miller MM, Provence SM, editors. *ASAM Criteria Treatment Criteria for Addictive, Substance-Related, and Co-Occurring Conditions*. 3rd ed. Carson City, NV: The Change Companies; 2013:69-104. [Context Link 1, 2, 3, 4, 5]
35. Fong T. Assessment. In: Brady KT, Levin FR, Galanter M, Kleber HD, editors. *Textbook of Substance Use Disorder Treatment*. 6th ed. American Psychiatric Association Publishing; 2021:63-76. [Context Link 1]
36. Kober D, Martin A. Inpatient psychiatric, partial hospital, and residential treatment for children and adolescents. In: Sadock BJ, Sadock VA, Ruiz P, editors. *Kaplan and Sadock's Comprehensive Textbook of Psychiatry*. 10th ed. Philadelphia, PA: Lippincott Williams & Wilkins; 2017:3797-3803. [Context Link 1, 2]
37. Nitzberg M, Smereck J. Diarrhea. In: Walls RM, editor. *Rosen's Emergency Medicine: Concepts and Clinical Practice*. 10th ed. Philadelphia, PA 19103-2899: Elsevier; 2023:245-251.e2. [Context Link 1]
38. Murphy SM, Irving CB, Adams CE, Waqar M. Crisis intervention for people with severe mental illnesses. *Cochrane Database of Systematic Reviews* 2015, Issue 12. Art. No.: CD001087. DOI: 10.1002/14651858.CD001087.pub5. [Context Link 1] View abstract...

39. Marshall M, Crowther R, Sledge WH, Rathbone J, Soares-Weiser K. Day hospital versus admission for acute psychiatric disorders. Cochrane Database of Systematic Reviews 2011, Issue 12. Art. No.: CD004026. DOI: 10.1002/14651858.CD004026.pub2. [Context Link 1] View abstract...
40. Baron DA, Cobb RT, Juarez GM. Other psychiatric emergencies. In: Sadock BJ, Sadock VA, Ruiz P, editors. Kaplan and Sadock's Comprehensive Textbook of Psychiatry. 10th ed. Philadelphia, PA: Lippincott Williams & Wilkins; 2017:2622-2637. [Context Link 1]

Footnotes

[A] The purpose of the care guidelines is to promote evidence-based care across the continuum of care to enhance the delivery of quality healthcare. Indications are presented for different levels of care. These indications help define the optimal level of care and can assist in developing alternatives to higher levels of care, tracking patient progress during treatment within a level of care, facilitating the progress of patients whose recovery is delayed, and preparing comprehensive plans for transition of patients from one level of care to another. Relevant professional society guidelines are foundational content for evaluation and treatment of behavioral health disorders at different levels of care and are complemented by the best available published evidence.(1)(2)(3)(4)(5)(6)(7)(8)(9) [A in Context Link 1]

[B] The Clinical Institute Withdrawal Assessment tool (CIWA) is used to assess 10 common withdrawal signs and symptoms, including: nausea/vomiting, tremor, anxiety, agitation, paroxysmal sweats; orientation and clouding of sensorium; tactile, auditory, and visual disturbances; and headache. The cumulative score is used to guide treatment management for patients undergoing withdrawal, and ranges from 0 to 67. Withdrawal severity may be categorized by score: mild (less than 10), moderate (10 to 18), severe (19 or greater with the absence of seizure, delirium, or hallucinations), or complicated (19 or greater with the presence of seizure, delirium, or new-onset hallucinations).(18)(19)(20) [B in Context Link 1]

[C] The Clinical Opiate Withdrawal Scale (COWS) is used by clinicians to measure 11 commonly observed opiate withdrawal signs or symptoms, including: resting pulse, sweating, restlessness, pupil size, bone or joint aches, runny nose or tearing, gastrointestinal upset, tremor, yawning, anxiety or irritability, and gooseflesh skin. Cumulative scores are described as mild (5 to 12), moderate (13 to 24), moderately severe (25 to 36), and severe (greater than 36). The COWS was developed to guide buprenorphine induction, and can be used to track opiate withdrawal symptoms over time or in response to treatment.(21)(22) [C in Context Link 1]

[D] Observation care can be provided in a psychiatric emergency services unit, holding unit, or other facility-based setting; or in a dedicated observation care unit or other hospital-based setting.(28)(29) [D in Context Link 1]

[E] Biopsychosocial stressors may impact the level of care necessary to manage a psychiatric or behavioral condition, including the ability of the program to meet comprehensive patient needs, ensure treatment adherence, enhance motivation, or prevent relapse (ie, comorbidities, environmental factors, or other barriers may prevent effective treatment at a less intensive level of care than might otherwise be appropriate to the patient's condition). Biopsychosocial assessment factors should be incorporated into care planning, including planned treatment goals, and intensity and duration of interventions. A "whole person care" approach that coordinates care across physical health, behavioral health, and social service sectors may bridge the gap of physical and mental health.(30)(31)(32)(33) Any identified deficits should be manageable by the program directly or through alternative arrangements.(1)(2)(5)(6) [E in Context Link 1]

[F] Comorbid conditions may directly impact the experience of psychiatric symptoms (eg, COPD and anxiety), or may indirectly impact determination of the appropriate venue for care (eg, routine blood sugar and insulin management in people with diabetes). If clinically appropriate, testing related to diagnosis or management (eg, screening for liver and kidney function, hepatitis, HIV, syphilis, tuberculosis) may be performed off-site.(1)(2)(5)(6) Assessment and treatment of co-occurring medical and/or developmental conditions through services and treatment settings capable of rendering integrated care is recommended.(1)(2)(4)(5)(6)(7) A "whole person care" approach that coordinates care across physical health, behavioral health, and social service sectors may bridge the gap of physical and mental health.(30)(31)(32)(33) The level that comorbid medical and/or developmental conditions are present may be described on a continuum (none/absent, low, moderate, and severe) and may impact determination of the appropriate level of care for treatment (ie, admission to a higher level of care).(1)(2)(5)(6)(34) [F in Context Link 1]

[G] Evaluation/assessment and treatment of co-occurring substance use disorders through services and treatment settings capable of rendering integrated care is recommended.(1)(2)(4)(5)(6) A "whole person care" approach that coordinates care across physical health, behavioral health, and social service sectors may bridge the gap of physical and mental health.(30)(31)(32)(33) The level that comorbid substance use disorders contribute to the primary presenting condition may be described on a continuum (none/absent, low, moderate, and severe) and may impact determination of the appropriate level of care for treatment (ie, admission to a higher level of care or specialty dual-diagnosis program).(1)(2)(5)(6)(34) [G in Context Link 1]

[H] The degree of environmental stressors and amount of support in the patient recovery environment should be considered in the context of the clinical presentation in determining the appropriate level of care for treatment. The level of environmental stressors may be low, mild, moderate, high, or extreme. The level of support in the environment may range from absent or minimal to limited to supportive or highly supportive.(1)(2)(4)(5)(6)(7)(34) [H in Context Link 1]

[I] Participation motivated by a wish to avoid negative consequences rather than accept the need to work toward recovery may require more intense monitoring and follow-up.(3)(17)(34) The patient's level of engagement may be described on a continuum: optimal, positive, limited, minimal, or unengaged. Readiness to change may range from actively and willingly engaged to unable to follow treatment recommendations due to clinical condition.(1)(2)(5)(6)(34) [I in Context Link 1]

[J] See Admission Guidelines in this guideline. [J in Context Link 1]

[K] The purpose of observation care is to provide time for patient testing, monitoring, or treatment that will help determine whether inpatient care is warranted. In this way, observation care is to be used for patients who are "close calls" in terms of need for admission to a specific behavioral health care setting. Appropriate use of MCG content presumes that observation care duration is finite, and that failure of a patient to improve sufficiently within the given time indicates appropriateness for admission to a specific behavioral health care setting.(37) Short-term crisis stabilization can be accomplished by crisis intervention care that can be done in a facility setting or by outreach teams.(38) For voluntary patients with acute manifestations of psychiatric disorders who are being considered for inpatient admission because of clinical urgency but are not expected to require physical restraint or around-the-clock nursing care, randomized controlled trials have shown that a residential setting can provide treatment that is no less effective than inpatient care.(39) [K in Context Link 1]

[L] Patients admitted to observation care with substance intoxication may be considered for discharge upon resolution of intoxication if follow-up evaluation finds symptoms have remitted sufficiently for safe discharge to an adequately supportive environment with appropriate follow-up plan, provided that risk of withdrawal can be effectively managed/treated at an available lower level of care.(24) (40) [L in Context Link 1]

Definitions

Care coordination

- Care coordination includes communication with **ALL** of the following(1)(2):
 - Outside psychiatrist, other therapists, and medical care providers as appropriate
 - Patient, family, guardian, and other supports as appropriate
 - All team members in other disciplines involved in ongoing patient care

References

1. Geller JL. The role of the hospital in the care of persons with mental illness. In: Sadock BJ, Sadock VA, Ruiz P, editors. Kaplan and Sadock's Comprehensive Textbook of Psychiatry. 10th ed. Philadelphia, PA: Lippincott Williams & Wilkins; 2017:4325-4346.
2. Kober D, Martin A. Inpatient psychiatric, partial hospital, and residential treatment for children and adolescents. In: Sadock BJ, Sadock VA, Ruiz P, editors. Kaplan and Sadock's Comprehensive Textbook of Psychiatry. 10th ed. Philadelphia, PA: Lippincott Williams & Wilkins; 2017:3797-3803.

Command auditory hallucinations

- An auditory hallucination is a "false perception of sound." This is the most common type of hallucination in psychiatric disorders and typically consists of voices but also may consist of any type of sound. A command auditory hallucination is a false perception of a voice giving orders to the patient that they may feel compelled to obey. Likely adherence to a command auditory hallucination is indicated by **1 or more** of the following(1)(2)(3)(4):
 - Perceived familiar or benevolent source
 - No belief by patient in their ability to control the voices
 - History of frequent response to such hallucinations with unquestioning obedience

References

1. Lewis SF, Escalona R, Keith SJ. Phenomenology of schizophrenia. In: Sadock BJ, Sadock VA, Ruiz P, editors. Kaplan and Sadock's Comprehensive Textbook of Psychiatry. 10th ed. Philadelphia, PA: Lippincott Williams & Wilkins; 2017:1406-1425.
2. Cohen CI, Izediuno I, Yadack AM, Ghosh B, Garrett M. Characteristics of auditory hallucinations and associated factors in older adults with Schizophrenia. American Journal of Geriatric Psychiatry 2014;22(5):442-9. DOI: 10.1016/j.jagp.2013.07.001.
3. Matorin AA, Shah AA, Ruiz P. Clinical manifestations of psychiatric disorders. In: Sadock BJ, Sadock VA, Ruiz P, editors. Kaplan and Sadock's Comprehensive Textbook of Psychiatry. 10th ed. Philadelphia, PA: Lippincott Williams & Wilkins; 2017:1114-1150.
4. Clementz BA, Keshavan MS, Pearson GD, Sweeney JA, Tamminga CA. Phenotypes of psychosis. In: Sadock BJ, Sadock VA, Ruiz P, editors. Kaplan and Sadock's Comprehensive Textbook of Psychiatry. 10th ed. Philadelphia, PA: Lippincott Williams & Wilkins; 2017:1512-1519.

Current plan

- Determining the existence of current plan for suicide, homicide, or serious harm to self or another involves clinical judgment regarding multiple factors. Higher risk may be indicated by a combination of factors including explicit or implicit evidence of **1 or more** of the following(1)(2):
 - Identification and/or availability of severe or lethal method (eg, using a gun)
 - Preparatory acts (eg, writing suicide note)
 - Intention that act will have severe or fatal outcome evidenced by planning where and when action will be taken (eg, act unlikely to be discovered and likely to be fatal)

References

1. Sudak HS. Suicide treatment. In: Sadock BJ, Sadock VA, Ruiz P, editors. Kaplan and Sadock's Comprehensive Textbook of Psychiatry. 10th ed. Philadelphia, PA: Lippincott Williams & Wilkins; 2017:2618-2622.
2. APA Work Group on Psychiatric Evaluation. The American Psychiatric Association practice Guidelines for the Psychiatric Evaluation of Adults. 3rd edition [Internet] American Psychiatric Association. 2016 Accessed at: <https://psychiatryonline.org/>. [accessed 2023 Sep 21] DOI: 10.1176/appi.books.9780890426760.

Discharge planning

- Discharge planning includes **ALL** of the following(1)(2)(3)(4)(5):
 - Identification of needed internal and external resources and services (eg, medical/care providers, care manager, housing, transportation, financial support)
 - Patient, family, guardian, and other supports involved in plan development as appropriate
 - Communication of current information to next level of care providers
 - Identification of contacts and their responsibilities at next level of care

References

1. Sowers WE, Rohland B. American Association of Community Psychiatrists' principles for managing transitions in behavioral health services. *Psychiatric Services* 2004;55(11):1271-5.
2. American Association of Community Psychiatrists. Level of Care Utilization System for Psychiatric and Addiction Services (LOCUS). Adult version 20 [Internet] American Association of Community Psychiatrists. 2016 Dec Accessed at: <https://www.communitypsychiatry.org/keystone-programs/locus>. [accessed 2023 Sep 11]
3. American Association of Community Psychiatrists. CALOCUS/CASII: Child and Adolescent Level of Care Utilization System. Child and Adolescent Version 20 [Internet] American Association of Community Psychiatrists. 2019 Jul Accessed at: <https://www.communitypsychiatry.org/keystone-programs/locus>. [accessed 2023 Sep 08]
4. CASII. Child and Adolescent Service Intensity Instrument [Internet] American Academy of Child and Adolescent Psychiatry. Accessed at: https://www.aacap.org/aacap/Member_Resources/Practice_Information/CASII.aspx. [accessed 2023 Sep 25]
5. Child & Adolescent Service Intensity Instrument (CASII) User's Manual version 4.0. American Academy of Child and Adolescent Psychiatry 2014 Oct.

Harm

- Harm to self or another is considered serious if it has a substantial likelihood of causing death, disability, or major disfigurement.(1)(2)(3)(4)(5)

References

1. American Association of Community Psychiatrists. Level of Care Utilization System for Psychiatric and Addiction Services (LOCUS). Adult version 20 [Internet] American Association of Community Psychiatrists. 2016 Dec Accessed at: <https://www.communitypsychiatry.org/keystone-programs/locus>. [accessed 2023 Sep 11]
2. American Association of Community Psychiatrists. CALOCUS/CASII: Child and Adolescent Level of Care Utilization System. Child and Adolescent Version 20 [Internet] American Association of Community Psychiatrists. 2019 Jul Accessed at: <https://www.communitypsychiatry.org/keystone-programs/locus>. [accessed 2023 Sep 08]
3. CASII. Child and Adolescent Service Intensity Instrument [Internet] American Academy of Child and Adolescent Psychiatry. Accessed at: https://www.aacap.org/aacap/Member_Resources/Practice_Information/CASII.aspx. [accessed 2023 Sep 25]
4. Child & Adolescent Service Intensity Instrument (CASII) User's Manual version 4.0. American Academy of Child and Adolescent Psychiatry 2014 Oct.
5. APA Work Group on Psychiatric Evaluation. The American Psychiatric Association practice Guidelines for the Psychiatric Evaluation of Adults. 3rd edition [Internet] American Psychiatric Association. 2016 Accessed at: <https://psychiatryonline.org/>. [accessed 2023 Sep 21] DOI: 10.1176/appi.books.9780890426760.

Imminent danger to others for adult

- Imminent danger to others for adult is present due to **1 or more** of the following(1)(2)(3):
 - Imminent risk for recurrence of attempt to seriously Harm another is present, as indicated by **ALL** of the following:
 - There has been a recent attempt to seriously Harm another.
 - There has not been Sufficient relief of factors that precipitated the attempt.
 - Current plan for homicide or serious Harm to another is present.
 - Command auditory hallucinations or paranoid delusions contributing to risk for homicide or serious Harm to another are present.
 - Patient has persistent thoughts of homicide or violent acts that likely could result in serious Harm to another that cannot be adequately monitored or treated at lower level of care, as indicated by **1 or more** of the following[A]:

- Necessary behavioral health care (such as required provider or lower level of care) not available or insufficient
- Patient support system inadequate
- Patient characteristics, such as extremely Impaired impulse control, judgment, or insight; severe Mania (adult), severe unreliability, or extreme agitation with desperation
- Ruminative flooding; uncontrollable and overwhelming negative thoughts
- Indication or report of repeated behavior, including physical or sexual aggression that is clearly injurious to others with history, plan or intent, and extremely Impaired impulse control, judgment, or insight

References

1. American Association of Community Psychiatrists. Level of Care Utilization System for Psychiatric and Addiction Services (LOCUS). Adult version 20 [Internet] American Association of Community Psychiatrists. 2016 Dec Accessed at: <https://www.communitypsychiatry.org/keystone-programs/locus>. [accessed 2023 Sep 11]
2. APA Work Group on Psychiatric Evaluation. The American Psychiatric Association practice Guidelines for the Psychiatric Evaluation of Adults. 3rd edition [Internet] American Psychiatric Association. 2016 Accessed at: <https://psychiatryonline.org/>. [accessed 2023 Sep 21] DOI: 10.1176/appi.books.9780890426760.
3. The Assessment and Management of Suicide Risk Work Group. VA/DoD Clinical Practice Guideline for the Assessment and Management of Patients at Risk for Suicide. VA/DoD Evidence Based Practice [Internet] U.S. Department of Veteran Affairs and the Department of Defense. Version 2.0; 2019 Accessed at: <https://www.healthquality.va.gov/guidelines/>. [accessed 2023 May 15]

Footnotes

- A. Patients with persistent thoughts of suicide, homicide, or serious harm to self or another may need frequent monitoring for progression to planning or intention to act on their thoughts. Such monitoring can be safely conducted at an outpatient level of care if providers and supports are adequately available and the patient can reliably participate in the monitoring process.(2)

Imminent danger to others for child or adolescent

- Imminent danger to others for child or adolescent is present due to **1 or more** of the following(1)(2)(3)(4)(5)(6):
 - Imminent risk for recurrence of attempt to seriously Harm another is present, as indicated by **ALL** of the following:
 - There has been a recent attempt to seriously Harm another.
 - There has not been Sufficient relief of factors that precipitated the attempt.
 - Current plan for homicide or serious Harm to another is present.
 - Command auditory hallucinations or paranoid delusions contributing to risk for homicide or serious Harm to another are present.
 - Patient has persistent thoughts of homicide or violent acts that likely could result in serious Harm to another that cannot be adequately monitored or treated at lower level of care, as indicated by **1 or more** of the following(A):
 - Necessary child or adolescent behavioral health care (such as required provider or lower level of care) not available or insufficient
 - Severe conflict in family environment or other inadequacy in patient support system
 - Patient characteristics, such as extremely Impaired impulse control, judgment, or insight; severe Mania (child or adolescent), severe unreliability, or extreme agitation with desperation
 - Ruminative flooding; uncontrollable and overwhelming negative thoughts
 - Indication or report of repeated behavior, including physical or sexual aggression that is clearly injurious to others with history, plan or intent, and extremely Impaired impulse control, judgment, or insight

References

1. Baron DA, Cobb RT, Juarez GM. Other psychiatric emergencies. In: Sadock BJ, Sadock VA, Ruiz P, editors. Kaplan and Sadock's Comprehensive Textbook of Psychiatry. 10th ed. Philadelphia, PA: Lippincott Williams & Wilkins; 2017:2622-2637.
2. American Association of Community Psychiatrists. CALOCUS/CASII: Child and Adolescent Level of Care Utilization System. Child and Adolescent Version 20 [Internet] American Association of Community Psychiatrists. 2019 Jul Accessed at: <https://www.communitypsychiatry.org/keystone-programs/locus>. [accessed 2023 Sep 08]
3. CASII. Child and Adolescent Service Intensity Instrument [Internet] American Academy of Child and Adolescent Psychiatry. Accessed at: https://www.aacap.org/aacap/Member_Resources/Practice_Information/CASII.aspx. [accessed 2023 Sep 25]
4. Child & Adolescent Service Intensity Instrument (CASII) User's Manual version 4.0. American Academy of Child and Adolescent Psychiatry 2014 Oct.
5. Newcorn JH, Kufert Y, Ivanov I, Chacko A. Aggression and violence. In: Dulcan MK, editor. Dulcan's Textbook of Child and Adolescent Psychiatry. 3rd ed. American Psychiatric Association Publishing; 2022:581-604.
6. The Assessment and Management of Suicide Risk Work Group. VA/DoD Clinical Practice Guideline for the Assessment and Management of Patients at Risk for Suicide. VA/DoD Evidence Based Practice [Internet] U.S. Department of Veteran Affairs and the Department of Defense. Version 2.0; 2019 Accessed at: <https://www.healthquality.va.gov/guidelines/>. [accessed 2023 May 15]
7. Sewall CJR, Goldstein TR, Brent DA. Youth suicide. In: Dulcan MK, editor. Dulcan's Textbook of Child and Adolescent Psychiatry. 3rd ed. American Psychiatric Association Publishing; 2022:551-564.

Footnotes

- A. Patients with persistent thoughts of suicide, homicide, or serious harm to self or another may need frequent monitoring for progression to planning or intention to act on their thoughts. Such monitoring can be safely conducted at an outpatient level of care if providers and supports are adequately available and the patient can reliably participate in the monitoring process.(7)

Imminent danger to self for adult

- Imminent danger to self for adult is present due to **1 or more** of the following(1)(2)(3):
 - Imminent risk for recurrence of Suicide attempt or act of serious Harm to self is present, as indicated by **ALL** of the following:
 - There has been a recent Suicide attempt or deliberate act of serious Harm to self.
 - There has not been Sufficient relief of the factors that precipitated attempt or act.
 - Current plan for suicide or serious Harm to self is present.
 - Command auditory hallucinations or paranoid delusions contributing to risk for suicide or serious Harm to self are present.
 - Patient has persistent Thoughts of suicide or serious Harm to self that cannot be adequately monitored or treated at lower level of care, as indicated by **1 or more** of the following[A]:
 - Necessary behavioral health care (such as required provider or lower level of care) not available or insufficient
 - Patient support system inadequate
 - Patient characteristics, such as extremely Impaired impulse control, judgment, or insight; severe Mania (adult), severe unreliability, or extreme agitation with desperation
 - Ruminative flooding; uncontrollable and overwhelming negative thoughts
 - Frantic hopelessness; fatalistic conviction that life will not improve, along with oppressive sense of entrapment and doom

References

1. American Association of Community Psychiatrists. Level of Care Utilization System for Psychiatric and Addiction Services (LOCUS). Adult version 20 [Internet] American Association of Community Psychiatrists. 2016 Dec Accessed at: <https://www.communitypsychiatry.org/keystone-programs/locus>. [accessed 2023 Sep 11]
2. APA Work Group on Psychiatric Evaluation. The American Psychiatric Association practice Guidelines for the Psychiatric Evaluation of Adults. 3rd edition [Internet] American Psychiatric Association. 2016 Accessed at: <https://psychiatryonline.org/>. [accessed 2023 Sep 21] DOI: 10.1176/appi.books.9780890426760.
3. The Assessment and Management of Suicide Risk Work Group. VA/DoD Clinical Practice Guideline for the Assessment and Management of Patients at Risk for Suicide. VA/DoD Evidence Based Practice [Internet] U.S. Department of Veteran Affairs and the Department of Defense. Version 2.0; 2019 Accessed at: <https://www.healthquality.va.gov/guidelines/>. [accessed 2023 May 15]

Footnotes

- A. Patients with persistent thoughts of suicide, homicide, or serious harm to self or another may need frequent monitoring for progression to planning or intention to act on their thoughts. Such monitoring can be safely conducted at an outpatient level of care if providers and supports are adequately available and the patient can reliably participate in the monitoring process.(2)

Imminent danger to self for child or adolescent

- Imminent danger to self for child or adolescent is present due to **1 or more** of the following(1)(2)(3)(4)(5)(6):
 - Imminent risk for recurrence of Suicide attempt or act of serious Harm to self is present, as indicated by **ALL** of the following:
 - There has been a recent Suicide attempt or deliberate act of serious Harm to self.
 - There has not been Sufficient relief of the factors that precipitated attempt or act.
 - Current plan for suicide or serious Harm to self is present.
 - Command auditory hallucinations or paranoid delusions contributing to risk for suicide or serious Harm to self are present.
 - Patient has persistent Thoughts of suicide or serious Harm to self that cannot be adequately monitored or treated at lower level of care, as indicated by **1 or more** of the following[A]:
 - Necessary child or adolescent behavioral health care (such as required provider or lower level of care) not available or insufficient
 - Severe conflict in family environment or other inadequacy in patient support system
 - Patient characteristics, such as extremely Impaired impulse control, judgment, or insight; severe Mania (child or adolescent), severe unreliability, or extreme agitation with desperation
 - Ruminative flooding; uncontrollable and overwhelming negative thoughts
 - Frantic hopelessness; fatalistic conviction that life will not improve, along with oppressive sense of entrapment and doom

References

1. American Association of Community Psychiatrists. CALOCUS/CASII: Child and Adolescent Level of Care Utilization System. Child and Adolescent Version 20 [Internet] American Association of Community Psychiatrists. 2019 Jul Accessed at:

<https://www.communitypsychiatry.org/keystone-programs/locus>. [accessed 2023 Sep 08]

2. CASII. Child and Adolescent Service Intensity Instrument [Internet] American Academy of Child and Adolescent Psychiatry. Accessed at: https://www.aacap.org/aacap/Member_Resources/Practice_Information/CASII.aspx. [accessed 2023 Sep 25]
3. Child & Adolescent Service Intensity Instrument (CASII) User's Manual version 4.0. American Academy of Child and Adolescent Psychiatry 2014 Oct.
4. Sewall CJR, Goldstein TR, Brent DA. Youth suicide. In: Dulcan MK, editor. Dulcan's Textbook of Child and Adolescent Psychiatry. 3rd ed. American Psychiatric Association Publishing; 2022:551-564.
5. The Assessment and Management of Suicide Risk Work Group. VA/DoD Clinical Practice Guideline for the Assessment and Management of Patients at Risk for Suicide. VA/DoD Evidence Based Practice [Internet] U.S. Department of Veteran Affairs and the Department of Defense. Version 2.0; 2019 Accessed at: <https://www.healthquality.va.gov/guidelines/>. [accessed 2023 May 15]
6. Shain B, Committee on Adolescence. Suicide and suicide attempts in adolescents. Pediatrics 2016;138(1):e20161420. DOI: 10.1542/peds.2016-1420. (Reaffirmed 2023 Jul)

Footnotes

- A. Patients with persistent thoughts of suicide, homicide, or serious harm to self or another may need frequent monitoring for progression to planning or intention to act on their thoughts. Such monitoring can be safely conducted at an outpatient level of care if providers and supports are adequately available and the patient can reliably participate in the monitoring process.(4)

Impaired impulse control, judgment, or insight

- Impaired impulse control describes a lack of control or ability to regulate actions related to inner urges. Impaired judgment and insight may include a lack of recognition of current or past symptoms, denial of need for treatment, or lack of recognition of consequences of actions. Levels of severity may be described as(1)(2)(3):
 - Minimal (subtle symptoms that may rarely bother the patient and are not associated with interference with daily functioning (may represent extreme end of normal range))
 - Mild (symptoms may not be very bothersome to the patient, are only occasionally present, and contribute to **Mild** dysfunction in daily living for adult or **Mild** dysfunction in daily living for child or adolescent)
 - Mild to moderate (symptoms may not be very bothersome to the patient, are established (eg, present up to half of each week), and contribute to **Mild to moderate** dysfunction in daily living for adult or **Mild to moderate** dysfunction in daily living for child or adolescent)
 - Moderate (symptoms may be somewhat bothersome to the patient, are clearly established (eg, present more than half of each week, but not daily or near daily), and contribute to **Moderate** dysfunction in daily living for adult or **Moderate** dysfunction in daily living for child or adolescent)
 - Moderately severe (symptoms may be bothersome to the patient, are present more than half of each week (but not daily or near daily), and contribute to **Serious** dysfunction in daily living for adult or **Serious** dysfunction in daily living for child or adolescent but are not all-consuming and usually can be contained at will)
 - Severe (symptoms may be very bothersome to the patient, are present daily (or near daily), and contribute to **Severe** dysfunction in daily living for adult or **Severe** dysfunction in daily living for child or adolescent (eg, symptoms may be all-consuming or cannot be contained at will, and often require close supervision))
 - Extreme (symptoms may be highly disturbing to the patient, are present on an almost constant basis, and contribute to **Severe** dysfunction in daily living for adult or **Severe** dysfunction in daily living for child or adolescent (condition usually requires close supervision))

References

1. Koita J, Riggio S, Jagoda A. The mental status examination in emergency practice. Emergency Medicine Clinics of North America 2010;28(3):439-51. DOI: 10.1016/j.emc.2010.03.008.
2. Baker SN. Management of acute agitation in the emergency department. Advanced Emergency Nursing Journal 2012 Oct-Dec;34(4):306-18; quiz 319-20. DOI: 10.1097/TME.0b013e31826f12d6.
3. Disruptive, impulse-control, and conduct disorders. In: American Psychiatric Association, editor. Diagnostic and Statistical Manual of Mental Disorders. DSM-5-TR ed. American Psychiatric Association; 2022:521-541.

Mania (adult)

- Mania symptoms may include elated/irritable mood, grandiosity, decreased need for sleep (eg, feeling rested after only a few hours of sleep), hyperverbal or rapid pressured speech, flight of ideas, racing thoughts, distractibility (ie, attention too easily drawn to unimportant or irrelevant external stimuli), increase in goal-directed activity (either socially, at work, or at school), hypersexuality, excessive energy, psychomotor agitation, or excessive involvement in pleasurable activities that have a high potential for negative consequences (eg, engaging in unrestrained buying sprees, sexual indiscretions, or foolish business investments). Symptoms do not have an alternative explanation (eg, intoxication, delirium, or other medical condition). Symptom severity may be described as(1)(2)(3)(4)(5)(6):

- Mild (periods of somewhat elevated, expansive, or irritable mood, or other manic symptoms that are occasionally present)
- Mild to moderate (periods of somewhat elevated, expansive, or irritable mood, or other manic symptoms that are present up to half the days of each week)
- Moderate (periods of extensively elevated, expansive, or irritable mood or restlessness, or other manic symptoms that are present more than half the days of each week)
- Moderately severe (frequent, but not daily, periods of extensively elevated, expansive, or irritable mood or restlessness, or other manic symptoms)
- Severe (daily periods of extensively elevated, expansive, or irritable mood or restlessness, or other manic symptoms)

References

1. Assessment measures. In: American Psychiatric Association, editor. Diagnostic and Statistical Manual of Mental Disorders. DSM-5-TR ed. American Psychiatric Association; 2022:841-857.
2. Altman EG, Hedeker D, Peterson JL, Davis JM. The Altman self-rating mania scale. *Biological Psychiatry* 1997;42(10):948-55. DOI: 10.1016/S0006-3223(96)00548-3.
3. Young RC, Biggs JT, Ziegler VE, Meyer DA. A rating scale for mania: reliability, validity and sensitivity. *British Journal of Psychiatry* 1978;133:429-35.
4. Altman Self-Rating Mania Scale. [Internet] Psychology Tools. Accessed at: <https://psychology-tools.com/test/altman-self-rating-mania-scale>. Updated 2023 [accessed 2023 Aug 30]
5. Young Mania Rating Scale. [Internet] Psychology Tools. Accessed at: <https://psychology-tools.com/test/young-mania-rating-scale>. Updated 2023 [accessed 2023 Sep 21]
6. Bipolar and related disorders. In: American Psychiatric Association, editor. Diagnostic and Statistical Manual of Mental Disorders. DSM-5-TR ed. American Psychiatric Association; 2022:139-175.

Mania (child or adolescent)

- Mania symptoms may include elated/irritable mood, grandiosity, decreased need for sleep (eg, feeling rested after only a few hours of sleep), hyperverbal or rapid pressured speech, flight of ideas, racing thoughts, distractibility (ie, attention too easily drawn to unimportant or irrelevant external stimuli), increase in goal-directed activity (eg, socially or at school), hypersexuality, excessive energy, psychomotor agitation, or excessive involvement in pleasurable activities that have a high potential for negative consequences. Symptoms do not have an alternative explanation (eg, intoxication, delirium, or other medical condition).(1)(2)

References

1. Pavuluri MN, Henry DB, Devineni B, Carbray JA, Birmaher B. Child mania rating scale: development, reliability, and validity. *Journal of the American Academy of Child and Adolescent Psychiatry* 2006;45(5):550-60. DOI: 10.1097/01.chi.0000205700.40700.50.
2. West AE, Celio CI, Henry DB, Pavuluri MN. Child Mania Rating Scale-Parent Version: a valid measure of symptom change due to pharmacotherapy. *Journal of Affective Disorders* 2011;128(1-2):112-9. DOI: 10.1016/j.jad.2010.06.013.

Mild dysfunction in daily living for adult

- **Mild** dysfunction in daily living for adult is present, as indicated by **1 or more** of the following(1)(2)(3)(4)(5)(6):
 - Some deterioration in interpersonal interactions (eg, increased conflict), but able to maintain some meaningful relationships
 - Some recent minor disruptions in self-care
 - Minor but consistent disruptions in social role functioning and meeting obligations, but still able to maintain these roles
 - Self-neglect with at least some minor limitation in ability to provide for basic needs (by self or others) in maintenance care
 - Other evidence of mild dysfunction

References

1. American Association of Community Psychiatrists. Level of Care Utilization System for Psychiatric and Addiction Services (LOCUS). Adult version 20 [Internet] American Association of Community Psychiatrists. 2016 Dec Accessed at: <https://www.communitypsychiatry.org/keystone-programs/locus>. [accessed 2023 Sep 11]
2. Assessment measures. In: American Psychiatric Association, editor. Diagnostic and Statistical Manual of Mental Disorders. DSM-5-TR ed. American Psychiatric Association; 2022:841-857.
3. WHO Disability Assessment Schedule 2.0 (WHODAS 2.0). 36-Item Instrument Scoring Sheet, Simple Scoring Calculation [Internet] World Health Organization. 2014 Nov Accessed at: <https://www.who.int/>. [accessed 2023 Aug 14]
4. WHO Disability Assessment Schedule 2.0 (WHODAS 2.0). 12-Item Instrument Scoring Sheet [Internet] World Health Organization. 2014 Nov Accessed at: <https://www.who.int/>. [accessed 2023 Aug 14]
5. Sheehan DV, Harnett-Sheehan K, Raj BA. The measurement of disability. *International Clinical Psychopharmacology* 1996;11 Suppl 3:89-95.
6. Sheehan Disability Scale (SDS). [Internet] Psychiatry & Behavioral Health Learning Network. 2019 Jan 28 Accessed at: <https://www.psychcongress.com/saundras-corner/scales-screeners/disability-scales/sheehan-disability-scale-sds>. [created 1983; accessed 2023 Aug 11]

Mild dysfunction in daily living for child or adolescent

- **Mild** dysfunction in daily living for child or adolescent is present, as indicated by **1 or more** of the following(1)(2)(3)(4)(5)(6)(7):
 - Minor deterioration or episodic failure in relationships with peers, family, or adults (eg, defiance, lying, provocative behavior)
 - Self-care sporadically below usual or expected standards
 - Impairments in function characterized by ability to show improvements following episodes of deterioration
 - Diminished ability to assess consequences of own actions (eg, acts of severe property damage) that therapeutic intervention can remediate
 - Self-neglect with at least some minor limitation in ability to provide for basic needs (by self or others) in maintenance care
 - Other evidence of mild dysfunction

References

1. American Association of Community Psychiatrists. CALOCUS/CASII: Child and Adolescent Level of Care Utilization System. Child and Adolescent Version 20 [Internet] American Association of Community Psychiatrists. 2019 Jul Accessed at: <https://www.communitypsychiatry.org/keystone-programs/locus>. [accessed 2023 Sep 08]
2. CASII. Child and Adolescent Service Intensity Instrument [Internet] American Academy of Child and Adolescent Psychiatry. Accessed at: https://www.aacap.org/aacap/Member_Resources/Practice_Information/CASII.aspx. [accessed 2023 Sep 25]
3. Child & Adolescent Service Intensity Instrument (CASII) User's Manual version 4.0. American Academy of Child and Adolescent Psychiatry 2014 Oct.
4. International Classification of Functioning, Disability and Health. Children & Youth Version [Internet] World Health Organization. 2007 Accessed at: <https://www.who.int/classifications/icf/en/>. [accessed 2022 Apr 28]
5. Shaffer D, et al. A children's global assessment scale (CGAS). Archives of General Psychiatry 1983;40(11):1228-31.
6. Lundh A, Kowalski J, Sundberg CJ, Gumpert C, Landen M. Children's Global Assessment Scale (CGAS) in a naturalistic clinical setting: Inter-rater reliability and comparison with expert ratings. Psychiatry Research 2010;177(1-2):206-10. DOI: 10.1016/j.psychres.2010.02.006.
7. Early Childhood Service Intensity Instrument (ECSII) for Infants, Toddlers, and Preschool-aged Children Ages 0-5 version 1. American Academy of Child and Adolescent Psychiatry 2009 Sep.

Mild to moderate dysfunction in daily living for adult

- **Mild to moderate** dysfunction in daily living for adult is present, as indicated by **1 or more** of the following(1)(2)(3)(4)(5)(6):
 - Some recent deterioration in interpersonal interactions (eg, conflicted, withdrawn), but maintains control of impulsive or abusive behaviors and maintains some meaningful relationships
 - Some recent disruptions in self-care below usual or expected standards
 - Disturbance in vegetative status (eg, eating, sleeping habits) that does not pose serious threat to health
 - Some deterioration in social role functioning and meeting obligations (eg, avoiding responsibilities on some occasions), but still can maintain roles overall
 - Other evidence of mild to moderate dysfunction

References

1. American Association of Community Psychiatrists. Level of Care Utilization System for Psychiatric and Addiction Services (LOCUS). Adult version 20 [Internet] American Association of Community Psychiatrists. 2016 Dec Accessed at: <https://www.communitypsychiatry.org/keystone-programs/locus>. [accessed 2023 Sep 11]
2. Assessment measures. In: American Psychiatric Association, editor. Diagnostic and Statistical Manual of Mental Disorders. DSM-5-TR ed. American Psychiatric Association; 2022:841-857.
3. WHO Disability Assessment Schedule 2.0 (WHODAS 2.0). 36-Item Instrument Scoring Sheet, Simple Scoring Calculation [Internet] World Health Organization. 2014 Nov Accessed at: <https://www.who.int/>. [accessed 2023 Aug 14]
4. WHO Disability Assessment Schedule 2.0 (WHODAS 2.0). 12-Item Instrument Scoring Sheet [Internet] World Health Organization. 2014 Nov Accessed at: <https://www.who.int/>. [accessed 2023 Aug 14]
5. Sheehan DV, Harnett-Sheehan K, Raj BA. The measurement of disability. International Clinical Psychopharmacology 1996;11 Suppl 3:89-95.
6. Sheehan Disability Scale (SDS). [Internet] Psychiatry & Behavioral Health Learning Network. 2019 Jan 28 Accessed at: <https://www.psychcongress.com/saundras-corner/scales-screeners/disability-scales/sheehan-disability-scale-sds>. [created 1983; accessed 2023 Aug 11]

Mild to moderate dysfunction in daily living for child or adolescent

- **Mild to moderate** dysfunction in daily living for child or adolescent is present, as indicated by **1 or more** of the following(1)(2)(3)(4)(5)(6)(7):
 - Some difficulty in relationships with peers, family, or adults (eg, defiance, lying), but without physical aggression
 - Self-care sometimes below usual or expected standards
 - Disturbance in vegetative status (eg, eating, sleeping habits) that does not pose serious threat to health

- Deteriorating school behavior such that disruption in school is threatened (eg, threat of expulsion)
- Diminished ability to assess consequences of own actions (eg, acts of severe property damage) that therapeutic intervention can remediate
- Other evidence of mild to moderate dysfunction

References

1. American Association of Community Psychiatrists. CALOCUS/CASII: Child and Adolescent Level of Care Utilization System. Child and Adolescent Version 20 [Internet] American Association of Community Psychiatrists. 2019 Jul Accessed at: <https://www.communitypsychiatry.org/keystone-programs/locus>. [accessed 2023 Sep 08]
2. CASII. Child and Adolescent Service Intensity Instrument [Internet] American Academy of Child and Adolescent Psychiatry. Accessed at: https://www.aacap.org/aacap/Member_Resources/Practice_Information/CASII.aspx. [accessed 2023 Sep 25]
3. Child & Adolescent Service Intensity Instrument (CASII) User's Manual version 4.0. American Academy of Child and Adolescent Psychiatry 2014 Oct.
4. International Classification of Functioning, Disability and Health. Children & Youth Version [Internet] World Health Organization. 2007 Accessed at: <https://www.who.int/classifications/icf/en/>. [accessed 2022 Apr 28]
5. Shaffer D, et al. A children's global assessment scale (CGAS). Archives of General Psychiatry 1983;40(11):1228-31.
6. Lundh A, Kowalski J, Sundberg CJ, Gumpert C, Landen M. Children's Global Assessment Scale (CGAS) in a naturalistic clinical setting: Inter-rater reliability and comparison with expert ratings. Psychiatry Research 2010;177(1-2):206-10. DOI: 10.1016/j.psychres.2010.02.006.
7. Early Childhood Service Intensity Instrument (ECSII) for Infants, Toddlers, and Preschool-aged Children Ages 0-5 version 1. American Academy of Child and Adolescent Psychiatry 2009 Sep.

Moderate dysfunction in daily living for adult

- **Moderate** dysfunction in daily living for adult is present, as indicated by **1 or more** of the following(1)(2)(3)(4)(5)(6):
 - Recent conflicted, withdrawn, or troubled relationships, but maintains control of impulsive, aggressive, or abusive behaviors
 - Self-care frequently below usual or expected standards
 - Significant disturbance in vegetative status (eg, eating, sleeping habits), but not posing serious threat to health
 - Significant deterioration in ability to fulfill responsibilities (eg, work, commitments to significant others), including neglect or avoidance of these responsibilities on some occasions
 - Ongoing and variably severe deficits in interpersonal relationships and ability to engage socially in constructive manner
 - Other evidence of moderate dysfunction

References

1. American Association of Community Psychiatrists. Level of Care Utilization System for Psychiatric and Addiction Services (LOCUS). Adult version 20 [Internet] American Association of Community Psychiatrists. 2016 Dec Accessed at: <https://www.communitypsychiatry.org/keystone-programs/locus>. [accessed 2023 Sep 11]
2. Assessment measures. In: American Psychiatric Association, editor. Diagnostic and Statistical Manual of Mental Disorders. DSM-5-TR ed. American Psychiatric Association; 2022:841-857.
3. WHO Disability Assessment Schedule 2.0 (WHODAS 2.0). 36-Item Instrument Scoring Sheet, Simple Scoring Calculation [Internet] World Health Organization. 2014 Nov Accessed at: <https://www.who.int/>. [accessed 2023 Aug 14]
4. WHO Disability Assessment Schedule 2.0 (WHODAS 2.0). 12-Item Instrument Scoring Sheet [Internet] World Health Organization. 2014 Nov Accessed at: <https://www.who.int/>. [accessed 2023 Aug 14]
5. Sheehan DV, Harnett-Sheehan K, Raj BA. The measurement of disability. International Clinical Psychopharmacology 1996;11 Suppl 3:89-95.
6. Sheehan Disability Scale (SDS). [Internet] Psychiatry & Behavioral Health Learning Network. 2019 Jan 28 Accessed at: <https://www.psychcongress.com/saundras-corner/scales-screeners/disability-scales/sheehan-disability-scale-sds>. [created 1983; accessed 2023 Aug 11]

Moderate dysfunction in daily living for child or adolescent

- **Moderate** dysfunction in daily living for child or adolescent is present, as indicated by **1 or more** of the following(1)(2)(3)(4)(5)(6)(7):
 - Troubled relationships with peers, family, or adults (eg, conflicted, withdrawn), but without physical aggression
 - Self-care frequently below usual or expected standards
 - Significant disturbance in vegetative status (eg, eating, sleeping habits), but not posing serious threat to health
 - Deteriorated school behavior such that disruption in school has occurred (eg, suspension, expulsion), and child is at risk for placement in alternative school setting or repeating grade
 - Diminished ability to assess consequences of own actions (eg, acts of severe property damage)
 - Other evidence of moderate dysfunction

References

1. American Association of Community Psychiatrists. CALOCUS/CASII: Child and Adolescent Level of Care Utilization System. Child and Adolescent Version 20 [Internet] American Association of Community Psychiatrists. 2019 Jul Accessed at: <https://www.communitypsychiatry.org/keystone-programs/locus>. [accessed 2023 Sep 08]
2. CASII. Child and Adolescent Service Intensity Instrument [Internet] American Academy of Child and Adolescent Psychiatry. Accessed at: https://www.aacap.org/aacap/Member_Resources/Practice_Information/CASII.aspx. [accessed 2023 Sep 25]
3. Child & Adolescent Service Intensity Instrument (CASII) User's Manual version 4.0. American Academy of Child and Adolescent Psychiatry 2014 Oct.
4. International Classification of Functioning, Disability and Health. Children & Youth Version [Internet] World Health Organization. 2007 Accessed at: <https://www.who.int/classifications/icf/en/>. [accessed 2022 Apr 28]
5. Shaffer D, et al. A children's global assessment scale (CGAS). Archives of General Psychiatry 1983;40(11):1228-31.
6. Lundh A, Kowalski J, Sundberg CJ, Gumpert C, Landen M. Children's Global Assessment Scale (CGAS) in a naturalistic clinical setting: Inter-rater reliability and comparison with expert ratings. Psychiatry Research 2010;177(1-2):206-10. DOI: 10.1016/j.psychres.2010.02.006.
7. Early Childhood Service Intensity Instrument (ECSII) for Infants, Toddlers, and Preschool-aged Children Ages 0-5 version 1. American Academy of Child and Adolescent Psychiatry 2009 Sep.

Serious dysfunction in daily living for adult

- **Serious** dysfunction in daily living for adult is present, as indicated by **1 or more** of the following(1)(2)(3)(4)(5)(6):
 - Serious deterioration in quality of interpersonal interactions (eg, impulsive or abusive behaviors)
 - Significant withdrawal and avoidance of almost all social interaction
 - Consistent failure to achieve self-care (eg, personal hygiene, appearance) to usual standards
 - Serious disturbance in vegetative status (eg, weight change, sleep disruption) threatening physical function
 - Inability to perform close to usual standards in adult obligations (eg, work, parenting) and neglecting these responsibilities frequently or over extended period of time
 - Other evidence of serious dysfunction

References

1. American Association of Community Psychiatrists. Level of Care Utilization System for Psychiatric and Addiction Services (LOCUS). Adult version 20 [Internet] American Association of Community Psychiatrists. 2016 Dec Accessed at: <https://www.communitypsychiatry.org/keystone-programs/locus>. [accessed 2023 Sep 11]
2. Assessment measures. In: American Psychiatric Association, editor. Diagnostic and Statistical Manual of Mental Disorders. DSM-5-TR ed. American Psychiatric Association; 2022:841-857.
3. WHO Disability Assessment Schedule 2.0 (WHODAS 2.0). 36-Item Instrument Scoring Sheet, Simple Scoring Calculation [Internet] World Health Organization. 2014 Nov Accessed at: <https://www.who.int/>. [accessed 2023 Aug 14]
4. WHO Disability Assessment Schedule 2.0 (WHODAS 2.0). 12-Item Instrument Scoring Sheet [Internet] World Health Organization. 2014 Nov Accessed at: <https://www.who.int/>. [accessed 2023 Aug 14]
5. Sheehan DV, Harnett-Sheehan K, Raj BA. The measurement of disability. International Clinical Psychopharmacology 1996;11 Suppl 3:89-95.
6. Sheehan Disability Scale (SDS). [Internet] Psychiatry & Behavioral Health Learning Network. 2019 Jan 28 Accessed at: <https://www.psychcongress.com/saundras-corner/scales-screeners/disability-scales/sheehan-disability-scale-sds>. [created 1983; accessed 2023 Aug 11]

Serious dysfunction in daily living for child or adolescent

- **Serious** dysfunction in daily living for child or adolescent is present, as indicated by **1 or more** of the following(1)(2)(3)(4)(5)(6)(7):
 - Serious deterioration in interpersonal interactions (eg, impulsive or abusive behaviors)
 - Significant withdrawal and avoidance of almost all social interaction
 - Consistent failure to achieve self-care as appropriate to age or developmental level
 - Serious disturbance in vegetative status (eg, weight change, sleep disruption) threatening physical function
 - Inability to perform adequately in school (including specialized setting) due to disruptive or aggressive behavior
 - Severely diminished ability to assess consequences of own actions (eg, acts of severe property damage)
 - Other evidence of serious dysfunction

References

1. American Association of Community Psychiatrists. CALOCUS/CASII: Child and Adolescent Level of Care Utilization System. Child and Adolescent Version 20 [Internet] American Association of Community Psychiatrists. 2019 Jul Accessed at: <https://www.communitypsychiatry.org/keystone-programs/locus>. [accessed 2023 Sep 08]
2. CASII. Child and Adolescent Service Intensity Instrument [Internet] American Academy of Child and Adolescent Psychiatry. Accessed at: https://www.aacap.org/aacap/Member_Resources/Practice_Information/CASII.aspx. [accessed 2023 Sep 25]
3. Child & Adolescent Service Intensity Instrument (CASII) User's Manual version 4.0. American Academy of Child and Adolescent Psychiatry 2014 Oct.

4. International Classification of Functioning, Disability and Health. Children & Youth Version [Internet] World Health Organization. 2007 Accessed at: <https://www.who.int/classifications/icf/en/>. [accessed 2022 Apr 28]
5. Shaffer D, et al. A children's global assessment scale (CGAS). Archives of General Psychiatry 1983;40(11):1228-31.
6. Lundh A, Kowalski J, Sundberg CJ, Gumpert C, Landen M. Children's Global Assessment Scale (CGAS) in a naturalistic clinical setting: Inter-rater reliability and comparison with expert ratings. Psychiatry Research 2010;177(1-2):206-10. DOI: 10.1016/j.psychres.2010.02.006.
7. Early Childhood Service Intensity Instrument (ECSII) for Infants, Toddlers, and Preschool-aged Children Ages 0-5 version 1. American Academy of Child and Adolescent Psychiatry 2009 Sep.

Severe dysfunction in daily living for adult

- **Severe** dysfunction in daily living for adult is present, as indicated by **1 or more** of the following(1)(2)(3)(4)(5)(6):
 - Incapacitation because of grave disability (eg, severe regression with inability to provide for self)
 - Extreme deterioration in social interactions (eg, threatening behaviors with little or no provocation)
 - Complete withdrawal from all social interactions
 - Complete neglect of self-care with associated impairment in physical status
 - Extreme disruption in vegetative function (eg, life-sustaining functions such as eating)
 - Complete inability to maintain any appropriate aspect of personal responsibility in any adult roles (eg, occupational, parental)
 - Other evidence of severe dysfunction

References

1. American Association of Community Psychiatrists. Level of Care Utilization System for Psychiatric and Addiction Services (LOCUS). Adult version 20 [Internet] American Association of Community Psychiatrists. 2016 Dec Accessed at: <https://www.communitypsychiatry.org/keystone-programs/locus>. [accessed 2023 Sep 11]
2. Assessment measures. In: American Psychiatric Association, editor. Diagnostic and Statistical Manual of Mental Disorders. DSM-5-TR ed. American Psychiatric Association; 2022:841-857.
3. WHO Disability Assessment Schedule 2.0 (WHODAS 2.0). 36-Item Instrument Scoring Sheet, Simple Scoring Calculation [Internet] World Health Organization. 2014 Nov Accessed at: <https://www.who.int/>. [accessed 2023 Aug 14]
4. WHO Disability Assessment Schedule 2.0 (WHODAS 2.0). 12-Item Instrument Scoring Sheet [Internet] World Health Organization. 2014 Nov Accessed at: <https://www.who.int/>. [accessed 2023 Aug 14]
5. Sheehan DV, Harnett-Sheehan K, Raj BA. The measurement of disability. International Clinical Psychopharmacology 1996;11 Suppl 3:89-95.
6. Sheehan Disability Scale (SDS). [Internet] Psychiatry & Behavioral Health Learning Network. 2019 Jan 28 Accessed at: <https://www.psychcongress.com/saundras-corner/scales-screeners/disability-scales/sheehan-disability-scale-sds>. [created 1983; accessed 2023 Aug 11]

Severe dysfunction in daily living for child or adolescent

- **Severe** dysfunction in daily living for child or adolescent is present, as indicated by **1 or more** of the following(1)(2)(3)(4)(5)(6)(7):
 - Incapacitation because of grave disability (eg, severe regression with inability to provide for self)
 - Extreme deterioration in interactions with peers, adults, or family (eg, assaultive behaviors with no provocation, high levels of family conflict)
 - Complete withdrawal from all social interactions
 - Complete neglect of self-care with associated impairment in physical status
 - Extreme disruption in vegetative function (eg, life-sustaining functions such as eating)
 - Nearly complete inability to maintain any appropriate school behavior or academic achievement
 - Severely diminished ability to assess consequences of own actions (eg, acts of severe property damage)
 - Other evidence of severe dysfunction

References

1. American Association of Community Psychiatrists. CALOCUS/CASII: Child and Adolescent Level of Care Utilization System. Child and Adolescent Version 20 [Internet] American Association of Community Psychiatrists. 2019 Jul Accessed at: <https://www.communitypsychiatry.org/keystone-programs/locus>. [accessed 2023 Sep 08]
2. CASII. Child and Adolescent Service Intensity Instrument [Internet] American Academy of Child and Adolescent Psychiatry. Accessed at: https://www.aacap.org/aacap/Member_Resources/Practice_Information/CASII.aspx. [accessed 2023 Sep 25]
3. Child & Adolescent Service Intensity Instrument (CASII) User's Manual version 4.0. American Academy of Child and Adolescent Psychiatry 2014 Oct.
4. International Classification of Functioning, Disability and Health. Children & Youth Version [Internet] World Health Organization. 2007 Accessed at: <https://www.who.int/classifications/icf/en/>. [accessed 2022 Apr 28]
5. Shaffer D, et al. A children's global assessment scale (CGAS). Archives of General Psychiatry 1983;40(11):1228-31.
6. Lundh A, Kowalski J, Sundberg CJ, Gumpert C, Landen M. Children's Global Assessment Scale (CGAS) in a naturalistic clinical setting: Inter-rater reliability and comparison with expert ratings. Psychiatry Research 2010;177(1-2):206-10. DOI:

10.1016/j.psychres.2010.02.006.

7. Early Childhood Service Intensity Instrument (ECSII) for Infants, Toddlers, and Preschool-aged Children Ages 0-5 version 1. American Academy of Child and Adolescent Psychiatry 2009 Sep.

Social Determinants of Health Assessment

- Risk of poor health outcomes may be increased by the presence of **1 or more** of the following social determinants of health(1)(2)(3)(4):
 - Housing insecurity, as indicated by **1 or more** of the following:
 - Individual or caregiver's current living situation is **1 or more** of the following(5):
 - Does not have own housing (eg, staying in a hotel, shelter, or with others)
 - Has own housing (eg, house, apartment), but at risk of losing it in the future (ie, behind on rent or mortgage)
 - Has own housing (eg, house, apartment), but has lived in 3 or more places in past year
 - Current housing has **1 or more** of the following:
 - Electrical appliances (eg, stove, refrigerator) not working or unavailable
 - Insufficient heating or cooling
 - Insufficient ventilation
 - Lead paint or pipes
 - Mold
 - Pests (eg, bugs) or rodents
 - Smoke detectors not working or unavailable
 - Food insecurity, as indicated by **1 or more** of the following(6):
 - In the past year, individual or caregiver ran out of food and did not have money to buy more food.
 - In the past year, individual or caregiver worried that they would run out of food before they received money to buy more food.
 - Insufficient transportation, as indicated by **1 or more** of the following(7):
 - In the past year, individual or caregiver missed medical appointments or could not get medications due to lack of transportation.
 - In the past year, individual or caregiver missed nonmedical activities, work, or could not get things needed for daily living due to lack of transportation.
 - Insufficient utilities, as indicated by **1 or more** of the following(8):
 - Utilities (eg, electricity, water, gas, or oil) are currently shut off or unavailable.
 - In the past year, electric, water, gas, or oil company threatened to shut off services.
 - Personal safety risk, as indicated by **2 or more** of the following(6):
 - Individual is sometimes or frequently physically hurt by another person (including family member).
 - Individual is sometimes or frequently insulted or talked down to by another person (including family member).
 - Individual is sometimes or frequently threatened with physical harm by another person (including family member).
 - Individual is sometimes or frequently screamed or cursed at by another person (including family member).
 - Insufficient dependent care, as indicated by **1 or more** of the following:
 - In the past year, individual or caregiver was unable to work due to lack of dependent care.
 - In the past year, individual or caregiver was unable to work more (additional) hours due to lack of dependent care.
 - In the past year, individual or caregiver missed medical appointments or could not get medications due to lack of dependent care.
 - In the past year, individual or caregiver missed nonmedical activities (eg, school, church, social activity) due to lack of dependent care.
 - Depression risk, as indicated by **ALL** of the following:
 - In the past 2 weeks, individual had little interest or pleasure in normal activities on at least several days.
 - In the past 2 weeks, individual felt down, depressed, or hopeless on at least several days.

References

1. Scanlon A, Reinisch C. Social determinants of health. In: Harding MM, Kwong J, Hagler D, Reinisch C, editors. *Lewis's Medical-Surgical Nursing: Assessment and Management of Clinical Problems*. 12th ed. St. Louis, MO: Mosby; 2023:18-20.
2. Moen M, Storr C, German D, Friedmann E, Johantgen M. A review of tools to screen for social determinants of health in the United States: a practice brief. *Population Health Management* 2020;23(6):422-429. DOI: 10.1089/pop.2019.0158.
3. Daniel-Robinson L, Moore JE. Innovation and Opportunities to Address Social Determinants of Health in Medicaid Managed Care. [Internet] Institute for Medicaid Innovation. 2019 Jan Accessed at: <https://medicaidinnovation.org/emerging-topics/>. [accessed 2023 Oct 20]
4. Billioux A, Verlander K, Anthony S, Alley D. Standardized Screening for Health-Related Social Needs in Clinical Settings: the Accountable Health Communities Screening Tool. [Internet] National Academy of Sciences. 2017 May Accessed at: <https://nam.edu/>. [accessed 2023 Sep 18]
5. Sandel M, et al. Unstable housing and caregiver and child health in renter families. *Pediatrics* 2018;14(2):e20172199. DOI: 10.1542/peds.2017-2199.

6. Children's HealthWatch Survey. Screening Instrument [Internet] Children's HealthWatch. 2020 Sep Accessed at: <https://childrenshealthwatch.org/>. [accessed 2023 Sep 11]
7. PRAPARE®: Protocol for Responding to and Assessing Patient Assets, Risks, and Experiences Screening Tool. [Internet] Association of Asian Pacific Community Health Organizations (AAPCHO) and National Association of Community Health Centers (NACHC). 2016 Sep Accessed at: <https://prapare.org/the-prapare-screening-tool/>. [accessed 2023 Sep 21]
8. Cook JT, et al. A brief indicator of household energy security: associations with food security, child health, and child development in US infants and toddlers. *Pediatrics* 2008;122(4):e867-75. DOI: 10.1542/peds.2008-0286.

Sufficient relief

- Sufficient relief is that which is adequate to alleviate the immediate impetus for recurrence of suicide attempt or act of harm to self or another.(1)(2)(3)

References

1. Sudak HS. Suicide treatment. In: Sadock BJ, Sadock VA, Ruiz P, editors. *Kaplan and Sadock's Comprehensive Textbook of Psychiatry*. 10th ed. Philadelphia, PA: Lippincott Williams & Wilkins; 2017:2618-2622.
2. Petti TA. Milieu treatment: inpatient, partial hospitalization, and residential programs. In: Dulcan MK, editor. *Dulcan's Textbook of Child and Adolescent Psychiatry*. 3rd ed. American Psychiatric Association Publishing; 2022:1019-1042.
3. APA Work Group on Psychiatric Evaluation. The American Psychiatric Association practice Guidelines for the Psychiatric Evaluation of Adults. 3rd edition [Internet] American Psychiatric Association. 2016 Accessed at: <https://psychiatryonline.org/>. [accessed 2023 Sep 21] DOI: 10.1176/appi.books.9780890426760.

Suicide attempt

- A suicide attempt may be identified by an actual, interrupted, or aborted act of self-injurious behavior accompanied by **1 or more** of the following(1)(2)(3):
 - Explicit or implicit evidence that the person believed the act would be lethal (eg, gaining access to gun, carbon monoxide inhalation)
 - Explicit or implicit evidence that the person intended to die (eg, act that was unlikely to be discovered in time for rescue, writing suicide note)

References

1. APA Work Group on Psychiatric Evaluation. The American Psychiatric Association practice Guidelines for the Psychiatric Evaluation of Adults. 3rd edition [Internet] American Psychiatric Association. 2016 Accessed at: <https://psychiatryonline.org/>. [accessed 2023 Sep 21] DOI: 10.1176/appi.books.9780890426760.
2. Sewall CJR, Goldstein TR, Brent DA. Youth suicide. In: Dulcan MK, editor. *Dulcan's Textbook of Child and Adolescent Psychiatry*. 3rd ed. American Psychiatric Association Publishing; 2022:551-564.
3. Sudak HS. Suicide treatment. In: Sadock BJ, Sadock VA, Ruiz P, editors. *Kaplan and Sadock's Comprehensive Textbook of Psychiatry*. 10th ed. Philadelphia, PA: Lippincott Williams & Wilkins; 2017:2618-2622.

Thoughts of suicide

- Thoughts of suicide or suicidal ideations are defined as "thoughts of serving as the agent of one's own death," to be distinguished from thoughts of death that do not involve actively bringing death about.(1)(2)

References

1. Sudak HS. Suicide treatment. In: Sadock BJ, Sadock VA, Ruiz P, editors. *Kaplan and Sadock's Comprehensive Textbook of Psychiatry*. 10th ed. Philadelphia, PA: Lippincott Williams & Wilkins; 2017:2618-2622.
2. APA Work Group on Psychiatric Evaluation. The American Psychiatric Association practice Guidelines for the Psychiatric Evaluation of Adults. 3rd edition [Internet] American Psychiatric Association. 2016 Accessed at: <https://psychiatryonline.org/>. [accessed 2023 Sep 21] DOI: 10.1176/appi.books.9780890426760.